Question to answer:

- Are we missing any discrete strategies within the groups that you believe supported NEST360 change and success?

Engage policy makers & leadership

- Obtaining buy-in for improving SSNC
- Advocate for funding and additional investment as needed
- Advocate for additional staff support and end mandatory staff rotations
- Advocating for official recognition of neonatal nursing as a specialized training
- Advocate for strengthened workshops for BMETs
- Development of standard, costed, neonatal unit floor plans
- Advocate for the hiring of a dedicated neonatal data clerk
- National TV, newspaper articles, announcements surrounding NEST and NEST personnel (communication strategies for reaching the wider public)
- Policy briefs
- Strengthen as needed” SSNC policies and guidelines, national SSNC targets (SDG and ENGAP targets and national commitments)
- Identify and engage local supply chains and engage medical supplies division of MOH
- Inclusion of ACS in national supply catalogues
- Engage with other key players in global SSNC (Beginnings Fund, mMamas, UNICEF, WHO, VAN)
- Collaboration with regulatory agencies?

QI activities

- Coordinate national and regional leadership to incorporate SSNC into existing QI structures (MOH quality directorates and policy frameworks)
- Developed training materials
- Integration of clinical and QI mentorship and training
- Development of standardized QI documentation and tools
- Development and dissemination of change package etc.
- Performed QI training
  - *When absent* developed a guide for how to set up QIST and WITs
  - *When inactive and routinely* QI coaching visits (in person and virtual zoom meeting)
- Development of communities of learning via WhatsApp
- QoC tools
- Recognition of positive staff performance
- Supporting the development of standardized clinical audit processes at the facility level
- Cross-country QI meetings
- Program and national QI capacity building within program leadership (Nebiyou to QI country leads)
- Foster QI projects occurring with active feedback and engagement from program team
- Incorporating IR tools and methods to accelerate the spread of change and develop generalizable learnings
- Communities of practice

Developing data systems to capture & feedback data and support routine SSNC data collection

- Service readiness checklist
- Health facility assessment
- NID
- Context tracker
- Hadli
- NEST-IT
- Patient capture tools (Admission forms, registers etc.)
- Developed guidelines for the use of data within NEST360
- Support integration of standard neonatal variables into national HMIS
- Utilize data clerks
- Identify and use SSNC relevant indicators (facility, regional, national, program)
- M&E reviews to inform programmatic decisions

Design & distribute clinical and biomedical educational resources

- Job aides
- Training manuals
- Guidelines (Decommission, workshop, SSNC, referral management, follow-up, installation trainings)
- Training videos
- GIC
- Case scenarios
- WASH, HH, IPC educational resources

Training and mentorship

- Virtual data review meetings
- Program wide data review meetings (ex. KIT)
- Data driven country implementation review meetings
- QI and data interpretation trainings
- Clinical care training (nationally approved in-depth SSNC courses for clinicians)
- BMET training (nationally approved in-depth courses)
- Facility-level mentorship (clinical coaching)
- Virtual/webinar CM/Nes
- Pre-service training (Integrated SSNC content in the clinical and BMET curricula, and ensure skills labs are set up for hands-on instruction)
- Inservice training (for BMETs and clinicians)
- Off site
    - Pre installation
    - On site low dose high frequency
    - Onsite induction course for new staff
    - Virtual training
- Developed and launched the process for peer-to-peer mentorship
- Developed and use communities of learning via WhatsApp
- PM and RM guidelines, decommissioning guidelines

Interprofessional collaboration & teamwork

- Fostering collaboration between clinicians, biomeds, data officers, and hospital leadership at the facility level
- Cross-team meetings at the program level (ex. KMC working groups, KITS and former Pause, Reflect, Act meetings)
- Collaboration with technical working groups (QI, BMET, newborn) within MOH at the country and regional/sub-regional levels
- Engagement of BMETs and labor unit in QI at the facility level
- Multidisciplinary training across cadres (doctors, biomeds, nurses) at the facility level
- SSNC Learning toolkit at the global level
- National pediatric associations at the country level
- Inclusion of BMETs in communities of practice at the program level

Motivate and empower staff

- Identifying and training SSNC champions
- Fostering collaborating between BMETs, clinicians (labor and newborn, leadership)

Ensure the availability of appropriate functional SSNC medical devices

- Providing devices to support hospital
- Providing consumables and spare parts to support hospitals
- Providing tool kits for biomeds
- Development of a consumables forecasting tool for the facility level
- Development of novel SSNC devices (Celsi warmer, monitor, biliDx, Pumani)
- Development of costing tool

Identify & qualify appropriate technology

- Development and continued refinement of TPPs
- Development and continued refinement of the innovations landscape and the newborn and maternal technology landscape
- NEST360 (PO4/5) engagement and collaboration with UNICEF/WHO engagement and collab (ask Becca)
- Development of the qualification and usability testing process
- Formation of NEST360 qualified technologies for SSNC
- Establishing budgets for tech management

Hatch Technologies

- Ensure consistent supply
- Financing mechanism
- Support for maintenance training

Engage family members in care process and follow up actions

- Training on providing parents with counseling
- Facilitating an enabling environment (culture, space, equipment etc.)
- Developed educational resources for parents (orientation to new families introduction to the NICU video, IPC, KMC, discharge, breast feeding education etc.)
- Encourage zero separation between the mother and newborn

Ensure adequate funding

- Supporting country teams in funding grants
- Identifying additional funding mechanisms globally and nationally
- Country-level costing and investment cases
- Encouraging facilities and MOHs to include budget lines on SSNC and BMETs (i.e., including infrastructure costs, human resources, running costs, and health insurance)

Question to answer:

- Do you agree with our rationale for how these strategy groupings support change?
  - If no, how do you believe the strategies influenced change?

Mapping mechanisms to strategy groups

Strategies used by NEST360

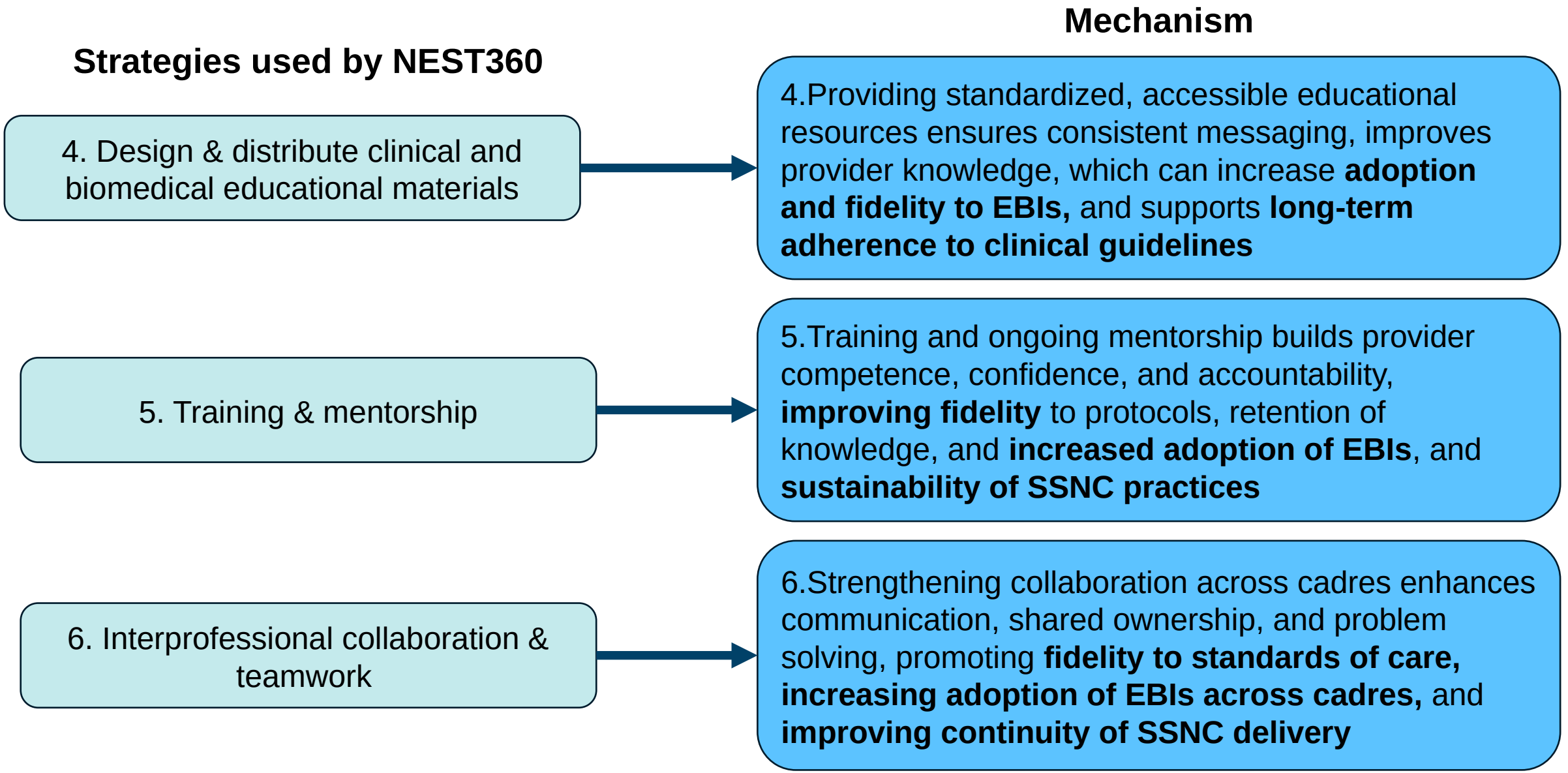
Mechanism

1. Engage policy makers & leadership → **1.** Policy-maker and leadership engagement fosters alignment of SSNC with national health priorities, **promotes long-term commitment,** and ensures **integration of SSNC into broader health system goals**

2. Quality improvement (QI) activities → 2.Routine data-driven QI cycles build a **culture of learning and accountability,** enabling timely identification of gaps and driving **sustainable improvements in SSNC coverage and quality**

3. Develop data systems to capture & feedback data and support routine SSNC data collection → 3.Robust, context-appropriate data systems enable **real-time monitoring**, feedback, and course correction, **supporting transparency,** and **improving responsiveness to local needs**

Detailed Strategies & Mechanisms: Continued



Detailed Strategies & Mechanisms: Continued

Strategies used by NEST360	Mechanism
7. Motivate staff and empower collaboration	7.Recognition, supportive supervision, and empowerment of staff **increases acceptability, motivation, and engagement** leading to greater accountability, reduced burnout, and **higher quality care delivery**
8. Ensuring the availability of appropriate functional SSNC medical devices	8.Ensuring appropriate, functional technology is available at the right level of care enables providers to delivery life-saving interventions safely and effectively, promoting **feasibility and adoption of EBIs**, and **reducing gaps in SSNC delivery**
9. Identify & qualify appropriate technology	9.Identifying, testing, and sharing results of devices that can function in rugged settings **increases the feasibility of their use to provide SSNC preventing device graveyards** and **increasing the adoption of SSNC EBIs** because devices remain functional and available

Detailed Strategies & Mechanisms: Continued

Strategies used by NEST360	Mechanism
10. Hatch Technologies (Africa-based technology distribution)	10.Reliable, context-appropriate technology distribution channels improve **equitable access to devices, reduce downtime** from broken or missing equipment, and promotes **sustainability of SSNC infrastructure in low-resource settings**
11. Engage family members in a care process and follow up actions	11.Actively involving families in care builds trust, **increases caregiver competence and adherence to recommended practices,** and **promotes continuity of care after discharge,** improving neonatal survival
12. Ensure adequate funding	12.Securing and sustaining funding streams enables consistent procurement of supplies, **maintenance of infrastructure,** and **retention of staff capacity,** ensuring **sustainability of SSNC services**
13. Tailoring local infrastructure	13.Tailoring local infrastructure creates **safer, more efficient care environments, improving feasibility, quality,** and **sustainability of SSNC services**

Tailoring local infrastructure

- Developing floor plans
- Provided small start-up grants to redesign flow or floor space or small updates
- Infrastructure fund (ex. Enabling back up power supply, and running water)